

WARNING I'm Testing Tirzepatide on Myself

Channel: Brandon Carter · Published: Jun 25, 2026 · Watch: <https://www.youtube.com/watch?v=EcbfOEwoq34> Source: full transcript

Executive Summary

Brandon Carter, nearly 43, already on TRT, announces a 30-day tirzepatide + peptide stack experiment aimed at hitting single-digit body fat without sacrificing muscle — documenting everything weekly as a live guinea pig for his audience.

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The Experiment Premise & Why He's Doing It 0:00

- First time ever using a GLP-1 / tirzepatide — treating himself as a "guinea pig" for his audience
 - Motivated by frequent viewer questions about peptides
 - Duration: **30 days**, with weekly video updates
- Core goal: lose body fat down to low single-digit body fat percentage while retaining — or ideally gaining — muscle
 - Has been lifting weights for nearly **three decades**, making muscle retention a meaningful benchmark
- Key concern with popular GLP-1 use (Ozempic, Wegovy, etc.): people lose weight fast but end up looking "skeletal"
 - Describes users losing muscle, bone density, and even facial muscle — "they look like zombies"
 - His solution: stack tirzepatide with other peptides specifically chosen to counter muscle loss
- Full disclosure: already on **testosterone replacement therapy (TRT)**, started just before age 40 (~4 years in)
 - Acknowledges viewers should factor TRT into how they interpret his results

Baseline Stats & Starting Point 1:52

- Age: will turn **43 in September**

- Weight: ~**203 lbs**
- Estimated body fat: ~**10%**
 - Put a shirtless photo into ChatGPT — it estimated 10%
 - InBody scan came back at 6%, which he dismisses as inaccurate ("we know that's bullshit")
- Target: **5–6% body fat** while maintaining or gaining muscle and strength
- Has been on a **ketogenic diet for 11 years** (notes one section as "8 years" for mitochondria adaptation — both figures mentioned)
 - Keto already substantially reduces his hunger — one reason he's not starting on an aggressive tirzepatide dose
 - Wore a **continuous glucose monitor for a full month**; the connected app thought he was fasting the entire time because his glucose was so stable — meaning minimal insulin spikes even when eating

The Full Peptide Stack 2:43

Peptide	Dose	Frequency	Timing	Primary Purpose
Tirzepatide	2.5 mg	Once per week	Not specified	Fat loss, appetite suppression, insulin sensitivity
Ipamorelin	200 mcg	5 days/week (weekdays only)	Before bed, fasted	Boost natural growth hormone, prevent catabolism
MOTS-c	1 mg (up to 5 mg)	3x/week (Mon/Wed/Fri)	Morning, fasted	Mitochondrial energy, counteract fatigue from caloric deficit
BPC-157	500 mcg	Daily	Timing flexible	Recovery, injury healing

- **Tirzepatide (2.5 mg/week)**
 - Deliberately low / conservative dose — "not aggressive because I'm not fat"
 - Mechanisms: suppresses hunger + improves insulin sensitivity "pretty dramatically"
 - Hunger suppression less critical for him given keto already handles that
 - Committed to staying at this dose and **not escalating** — wants to ensure he still eats enough protein
- **Ipamorelin (200 mcg, weekdays before bed)**
 - Stimulates natural growth hormone release
 - Rationale: GH should signal the body to burn fat rather than muscle during a caloric deficit (anti-catabolic)
 - Must be taken fasted — conflict with his habit of eating a large meal right before sleep
 - Plans to build in a grace period between last meal and injection
 - Less of a concern on keto since he doesn't get significant insulin spikes from his normal food — confirmed by CGM data
- **MOTS-c (1–5 mg, Mon/Wed/Fri mornings fasted)**

- Helps mitochondria extract more energy
- His mitochondria are already fat-adapted after years of keto — expects this to "supercharge" results and energy
- Key role: counteract the tiredness and low energy that commonly hits people using GLP-1s in a deep caloric deficit
- Starting at 1 mg, willing to go up to 5 mg based on how he feels
- **BPC-157 (500 mcg daily, any time)**
 - Nicknamed the "Wolverine peptide" for its recovery and healing properties
 - Personal motivation: decades of accumulated minor injuries — small persistent aches that don't stop him from training but never fully resolve
 - Wants to observe whether chronic minor pains actually disappear on BPC-157
- Also continuing **TRT** (testosterone replacement therapy) throughout — already established baseline

Diet & Protein Strategy 6:45

- Minimum **1 gram of protein per pound of bodyweight** (~200 g/day absolute floor)
 - Will track calories daily — has tracked every single day for ~**two decades** without exception
 - Not guessing; will measure and log
- If daily calories drop below **2,000**, will increase protein to **1.5 g per pound of bodyweight**
 - Based on past extreme cut experience: slashed calories very low, did **40,000 steps/day**, and successfully retained significantly more muscle by keeping protein at 1.5 g/lb
 - In that prior cut: ~**300 g protein/day** on only **1,000–1,500 total calories**
- If tirzepatide kills appetite and eating solid food becomes difficult, will rely on **protein shakes** — "even if I've got to drink like eight of these a day"
- Remaining on **ketogenic diet** throughout the experiment

Training Approach During the Cut 8:21

- Will continue lifting **heavy** — not switching to high-rep, low-rest "cutting" style workouts
 - Rationale: heavy lifting sends the signal to the body to retain muscle mass
 - "I probably lift heavier when I'm cutting"
- Rep ranges: **sets of ~6–8 reps**, resting as much as needed between sets
- Workout style: **full body workouts**, which he has done almost exclusively for over a decade
 - Primarily compound movements
 - Exceptions: bicep curls, tricep work, and calves — difficult to hit adequately with pure compounds

Documentation Plan & What to Expect 9:28

- **Weekly updates** for the full 30 days

- Will document:
 - Lift performance (strength numbers)
 - Sleep quality
 - Daily diet
 - Subjective feelings / energy / side effects
- Hypothesis:
 - **Likely outcome:** reaches single-digit body fat, retains majority of muscle
 - **Best case:** puts on a small amount of muscle simultaneously
- Has not injected anything yet at time of filming — this video is the announcement/protocol reveal
- Explicitly notes this is an **experiment** — he does not know how his body will react
- Invites audience feedback: peptide experts, critics, supporters all welcome to comment

"If I feel like shit, that's good information to have. If I feel fantastic because the MOTS-c gave me a ton of energy, that's good information to have as well."